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Examining the Views of Training Psychological Counselors on Trauma: A Qualitative Study

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Abstract

This study aimed to examine the views of training counselors working with trauma on the concept of trauma. In this context, training counselors were interviewed regarding the symptoms observed in traumatized individuals and their views on the post-traumatic recovery process. The study was conducted using the phenomenology design, a qualitative research method. Criterion sampling, one of the purposive sampling methods, was used to determine the study group. The study group consisted of 10 psychological counselors, 6 (60%) female and 4 (40%) male, who had received training in training in trauma and were working in public institutions. A semi-structured interview form was prepared by consulting field experts to collect data from the participants. The data obtained from the training counselors' views were analyzed using the content analysis technique. As a result of the study, a total of 5 themes were created regarding the symptoms observed in traumatized individuals: Physical and Behavioral Symptoms, Emotional Symptoms, Cognitive Symptoms, Psychological Symptoms, and Course of Symptoms. A total of six themes were generated regarding the views of training counselors on the post-traumatic recovery process: Stages of Recovery, Positive Influencing Factors, Negative Influencing Factors, Social Support Systems, Individual Strengths, and Individual Weaknesses. The results of the study were discussed in light of the relevant literature, and recommendations were included.

Key Words

Post-traumatic recovery • Psychological counselor • Training counselor • Trauma

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Introduction

Trauma is defined as events or experiences that threaten an individual's life or psychological integrity and make coping difficult in relational and emotional dimensions ([Herman, 2017](#)). Trauma is a disturbing experience that may result in helplessness, fear, dissociation, confusion or other intense emotions that can have a long-term negative impact on an individual's behavior, attitudes and other aspects of functioning. These experiences shake an individual's belief that the world is a safe, fair and predictable place ([American Psychological Association, 2024](#)). Traumatic experiences disrupt an individual's normal behavioral system by losing their power of control, the ability to form bonds, and their sense of the meaning of life ([Herman, 2017](#)). What makes a traumatic event extraordinary is not only its unexpectedness, but also the fact that it paralyzes the coping mechanisms that serve to adapt to life events ([Türksoy, 2003](#)). However, a traumatic event; It must be a sudden, uncontrollable, and unpreventable experience that suppresses the individual's capacity to respond ([Calhoun & Tedeschi, 1999](#)).

Traumatic experiences can arise from events such as losses, abuse, or neglect experienced at the individual level, but can also manifest as situations that affect larger communities, such as natural disasters, wars, and forced migrations ([Van der Kolk, 2014](#)). Challenging experiences are diverse, and it seems likely that most individuals will experience at least one traumatic experience in their lives ([Taylor, 2023](#)). A study conducted by [Benjet et al. \(2016\)](#), collecting data from 24 different countries, revealed that 70% of participants had encountered a traumatic experience at least once in their lifetime. Similarly, many studies in the literature show that traumatic experiences are common ([Al-Saffar et al., 2003](#); [Bakır, 2023](#); [Davidson, 2000](#); [Du et al., 2022](#); [Hepp et al., 2006](#); [Karancı et al., 2009](#); [Morrison, 2018](#); [Sarıoğlu, 2014](#); [Williams et al., 2007](#)). The prevalence of traumatic experiences can predispose individuals to the emergence of various psychological distress. In this regard, studies in the relevant literature indicate that some psychological distress and difficulties can be experienced after trauma ([Güloğlu & Karairmak, 2013](#)). For example, traumatic experiences are reported to be associated with depression ([Cénat et al., 2020](#); [Gül et al., 2016](#)), anxiety ([Cougler et al., 2010](#); [Kurapov et al., 2023](#); [Okçay & Es, 2024](#)), and post-traumatic stress disorder ([Akdağ et al., 2024](#); [Orçan & Karaaziz, 2024](#); [Rezayat et al., 2020](#)). In this sense, individuals may not be able to understand what they are feeling, may have difficulty expressing their emotions, and may be unable to exhibit adaptive responses when experiencing somatic problems as a result of undifferentiated emotions ([Şar, 2017](#)). Furthermore, exposure to trauma and the increase in post-traumatic stress disorder that develops after a traumatic event decrease individuals' quality of life ([Carlsson et al., 2006](#)). Therefore, considering the prevalence of trauma and its negative effects on individuals, protecting mental health is of paramount importance, and psychological counselors are considered to play a critical role in this process.

When people encounter self-harming experiences, their cognitive and emotional balance are negatively affected, and they seek help to preserve or reconstruct their personal integrity ([Gordon et al., 2000](#)). In this process, psychological counselors help individuals make sense of their difficulties and develop healthy coping strategies. The goal of the counseling process is for clients to recognize their opportunities and resources, use them more effectively, and become self-help individuals in their daily lives ([Egan, 2011](#)). Therefore, psychological counselors working with trauma, and especially professionals who assume a mentoring role in this field, bear a significant responsibility both in the individual counseling process and in the context of professional training and guidance. However, the literature on the experiences and perceptions of trainer psychological

counselors working in the field of trauma appears to be limited (Cingirit, 2024; Gönültaş & Savi Çakar, 2020; Parker & Henfield, 2012; Yavrutürk, 2023). This limitation restricts both the development of trauma-focused counseling practices and the improvement of professional training processes. It is believed that the experiences and observations of trainer psychological counselors have the potential to directly influence the training processes of their colleagues, their supervision needs, and the applicability of trauma-focused interventions. Consequently, this study aims to contribute to the literature by examining in depth the views of trainer psychological counselors on the concept of trauma and to strengthen professional practice and training processes. In this regard, the following questions were addressed.

a. Symptoms in Traumatized Individuals: (i) What are the most common physical and behavioral symptoms you encounter in individuals who have experienced trauma? Can you give examples?, (ii) What are the most common emotional symptoms you encounter in individuals who have experienced trauma? Can you give examples?, (iii) What are the most common cognitive symptoms you encounter in individuals who have experienced trauma? Can you give examples?, (iv), What are the most common psychological symptoms you encounter in individuals who have experienced trauma? Can you give examples?, (v)What can you say about the duration and intensity of trauma symptoms?, (vi)What differences in symptoms are observed depending on the type of trauma? Can you describe them?

b. Recovery Processes of Individuals Who Have Experienced Traumatization: (i) What stages do you typically observe in the post-trauma recovery process?, (ii) What factors positively affect recovery processes?, (iii) What factors negatively affect recovery processes?, (iv) How do you evaluate the role of social support systems (family, friends, and community) in the recovery process?, (v) What do you think are the most important factors that support the recovery of individuals who have experienced trauma? (Strengths), (vi) What do you think are the most important factors that complicate or delay the recovery of individuals who have experienced trauma? (Weaknesses)

Method

Research Design

This study aimed to examine the views of training psychological counselors on trauma. The research was conducted using the phenomenological approach, a qualitative research method. Phenomenology is an approach that allows individuals to express their perceptions, perspectives, feelings, and thoughts about certain concepts and is used to describe how they experience these phenomena (Rose et al., 1995). This study aimed to understand the views, experiences, and perceptions of training psychological counselors working with trauma through the participants' own statements.

Study Group

This study used the criterion sampling method, which is a purposive sampling method. Criterion sampling allows for the identification of individuals with characteristics that can represent a particular phenomenon (Büyüköztürk et al., 2016). Furthermore, criterion sampling makes it possible to study all situations that meet a set of predetermined criteria (Baltacı, 2018). In this study, psychological counselors who had received training in trauma training and worked in public institutions were selected as the criterion, and individuals who met this criterion were included in the study. The study participants consisted of 10 individuals in total, with 7 holding

undergraduate degrees and 3 holding graduate degrees. The participants received their education from various institutions and programs. Descriptive information about the participants is shown in Table 1.

Table 1

Descriptive information on the study group

Participant	Gender	Age	Educational Level	Years of Experience	Populations Studied in the Field of Trauma	Currently Working in the Field of Trauma
P1	Female	33	Bachelor's Degree	9	Child Adolescent	Yes
P2	Male	31	Bachelor's Degree	7	Child Adolescent	Yes
P3	Female	30	Bachelor's Degree	6	Child Adolescent Adult Disaster Victims	Yes
P4	Female	32	Bachelor's Degree	10	Child Adolescent Adult Disaster Victims	Yes
P5	Male	38	Bachelor's Degree	13	Child Adolescent Adult	Yes
P6	Female	30	Bachelor's Degree	6	Child Adolescent Disaster Victims	Yes
P7	Female	27	Master's Degree	5	Child Adolescent Disaster Victims	Yes
P8	Male	36	Bachelor's Degree	11	Adolescent Disaster Victims	Yes
P9	Female	30	Master's Degree	8	Child Adolescent	Yes
P10	Male	30	Master's Degree	5	Child Disaster Victims	Yes

As seen in Table 1, the study group consisted of 10 participants. In this study, face-to-face interviews were conducted with the participants. Smaller study groups are preferred in face-to-face interviews compared to the survey method (Karagöz, 2017). Therefore, the study group consisted of 10 people. To ensure the confidentiality of participant identities, the individuals in the study group were coded as K1, K2, and K10. The age of the participants ranged from 27 to 38, and the average age was calculated as $\bar{x}=31.7$.

Data Collection Tool

A semi-structured interview form was used as the data collection tool. First, the relevant literature on the research topic was reviewed, and the study questions were determined. Later, the draft of the form was created by obtaining the opinions of three faculty members from the field of guidance and psychological counseling. A pilot study was conducted with two individuals to assess the feasibility of the form, and the questions were adjusted based on the feedback obtained from this study, and the semi-structured interview form was finalized. This process aimed to ensure the validity of the form and its suitability for the research objectives.

Data Collection

Research data were collected during the last two weeks of November 2025 through individual interviews with psychological counselors who met the specified criteria and agreed to volunteer for the study. Participants were selected from among psychological counselors who work in public institutions and have received training in trauma training. Participant contact was primarily conducted by phone, and a face-to-face meeting was planned after determining the appropriate setting and time. Before the interviews, participants were provided detailed information about the purpose, content, and interview process, and an informed consent form was signed. Furthermore, informing participants that their identities will be kept confidential in the informed consent form increases the likelihood of sincere responses (Shaughnessy et al., 2016). The interviews lasted approximately 45-55 minutes. With the permission of the participants, the interviews were audio-recorded.

Data Analysis

The data obtained from the study were evaluated using content analysis. Content analysis is a method that aims to systematically examine collected data and reveal relationships and conceptual frameworks between them. In this process, similar data are organized under specific themes and categories. The findings are structured and interpreted in a way that allows the reader to make sense of them (Büyükoztürk et al., 2016).

The content analysis process in this study was conducted in accordance with Creswell and Creswell's (2017) four-stage approach to qualitative data analysis. Creswell and Creswell (2017) recommend that data be systematically processed, coded, and transformed into themes to make sense of qualitative research. In the first stage, the audio recordings obtained from the interviews were transcribed and analyzed. In the second stage, the written data were read and reviewed in detail to develop a holistic understanding. This process helped the researcher familiarize themselves with the data and develop a preliminary understanding of potential coding. In the third stage, the data were systematically coded; similar content and expressions were identified and grouped under specific codes. Following the completion of the coding, the fourth stage was moved on to create higher-level categories and themes by considering the relationships between the codes. In the final stage, these themes and categories were evaluated holistically, and the findings were supported by sample participant statements.

Validity and Reliability in Qualitative Research

This study employed a qualitative analysis method to conduct an in-depth examination of the views of training counselors working with trauma. The study aimed to verify the reliability of qualitative research using the Consensus/(Disagreement + Consensus) formula recommended by Miles and Huberman (1994). According to this formula, acceptable values for consensus are at least 80%. Calculations performed by three coders on 65 codes revealed 57 points of agreement and 8 points of disagreement. The formula resulted in an overall agreement rate of 88% for this study.

The study detailed the context and participant profile of the study. This increased the transferability of the findings to the relevant literature. Confirmability strategies were employed to mitigate the influence of researcher biases and ensure the objectivity of the data. In this context, interviews were conducted with three faculty members from the field of guidance and psychological counseling while collecting data and theme and category development. This approach allowed for the data collection, theme, and category development processes to be conducted in an auditable manner.

Results

The findings regarding the responses of the training psychological counselors to questions about trauma symptoms are given in Table 2.

Table 2

Findings regarding responses to questions about trauma symptoms by training psychological counselors working with trauma

Themes	Categories	Frequency	Percentage (%)
Physical and Behavioral Symptoms	Headache	6	%60
	Stomach Pain/Nausea	6	%60
	Avoidance	7	%70
	Sleep Disorder	6	%60
	Nail Biting	3	%30
	Incontinence	2	%20
	Tics	2	%20
	Hyperactivity	3	%30
	Attention Deficit	5	%50
	Unwillingness	8	%80
	Dullness	7	%70
	Anorexia	4	%40
	Aggression	2	%20
Emotional Symptoms	Anger	10	%100
	Sadness	10	%100
	Guilt	6	%60
	Inadequacy	5	%50
	Helplessness	6	%60
	Worthlessness	5	%50
	Hopelessness	4	%40
	Regret	5	%50
Cognitive Symptoms	Difficulty in Decision Making	6	%60
	Forgetfulness	7	%70
	Inability to Focus	5	%50
	Rumination	4	%40
	Self-Blame	5	%50
	Confusion	4	%40
Psychological Symptoms	Panic Attack	5	%50
	Depression	6	%60
	Somatization	4	%40
	Anxiety	7	%70
Course of Symptoms	Sudden Onset	6	%60
	Increase Over Time	3	%30
	Type of Trauma	6	%60
	Personal Characteristics	8	%80

Table 2 presents the findings regarding the views of trauma counselors working with trauma patients on trauma symptoms. Table 2 reveals a total of five themes: Physical and Behavioral Symptoms, Emotional Symptoms, Cognitive Symptoms, Psychological Symptoms, and Course of Symptoms. Below are the categories that comprise these themes and the direct participant opinions that support these categories.

Within the Physical and Behavioral Symptoms theme, 13 categories were generated: Headache, Stomach Pain/Nausea, Avoidance, Sleep Disorder, Nail Biting, Incontinence, Tics, Hyperactivity, Attention Deficit,

Unwillingness, Dullness, Anorexia, and Aggression. The following texts provide some of the views of training counselors on physical and behavioral trauma symptoms.

"There may be dullness, sleep problems, appetite problems, difficulty following routines, headaches, nausea, dizziness, attacks, palpitations, distraction, and a lack of desire to do anything." (P1)

"These people aren't very interested in life. I observe a lack of interest and introversion. I often see them as having no connection to life and not being able to enjoy it." (P2)

"Sometimes the person has difficulty sleeping because of their thoughts." (P4)

"They exhibit a freeze reaction. They are particularly dull. They have a hard time listening." (P5)

"Their hands are clenched or they clench. This clenching means they're closed. Their body is noticeably tight and rigid. They may have a frozen look. They can often appear in a freeze mode." (P7)

"Physically, nail biting occurs. Children, in particular, may wet their pants in the evenings, and behavioral hyperactivity can occur. Adolescents can develop certain tics. Adolescents can become more withdrawn. Attention deficits can also be more prevalent in children. Adolescents can become more withdrawn and self-centered." (P9)

"They reactively close themselves off from relationships." (P10)

Within the Emotional Symptoms theme, 8 categories were created: Anger, Sadness, Guilt, Inadequacy, Helplessness, Worthlessness, Hopelessness, and Regret. The following texts include some of the views of training counselors regarding symptoms of emotional trauma.

"Their emotions can be very high. They can feel extreme loneliness and helplessness." (P2).

"Because there's a dulling of emotion, we often observe anger-driven approaches. Instead of appropriate and realistic expression, they generalize their reactions based on the emotion they experienced at the time of the trauma." (P5)

"Because this is where feelings like anger and resentment are experienced. A sense of guilt also arises, as if they wish they were there." (P6)

"Frankly, reactions like worthlessness, inadequacy, loneliness, and humiliation often revolve around these." (P8)

"Adolescents can experience sudden mood swings. They can cry one moment and then return to normal the next. Children become more emotionally reserved. They become sad and don't talk. Adolescents' mood swings are faster. They suddenly become happy and then angry..." (P9)

"I think they are more cruel to themselves than they would be to anyone else. They are generally in a state of hopelessness." (P10)

Within the Cognitive Symptoms theme, 6 categories were generated: Difficulty in Decision Making, Forgetfulness, Inability to Focus, Rumination, Self-Blame, and Confusion. The following texts include some of the views of training psychological counselors regarding symptoms of cognitive trauma.

"They can't move on to other areas of life. They can't think about everyday things. Sometimes they get fixated on something related to a traumatic event in their past. Let's say someone was sexually abused. Why did I go to that house? Why did I talk to them? Did I seem too inviting? They keep thinking about these things cognitively." (P2)

"I observe an inability to stay in the moment, to focus on the present moment." (P3)

"They replay that event over and over in their mind." (P4)

"Human-induced traumas affect people more. There can be a reason or a cause for natural trauma. For example, a flood might have been caused by a lot of rain. An earthquake might have been caused by fault lines. But for example, with abuse, a person can't pinpoint it or make sense of it. Why did someone do this to me? Did they love me or not? If they did it out of love, why does it hurt so much? In other words, the person experiences confusion." (P8)

"Because they have temporary memory loss, more fragmented, partial memories can come to mind. This is why they have more distorted thoughts. Because they can't establish a connection, they can have difficulty grounding events logically." (P9)

"They often have automatic thoughts, and they drown in these automatic thoughts. Due to the multitude and intensity of these thoughts, they develop a very judgmental language toward themselves. These might be the language of their parents or other people older than themselves. More often, they are cruel to themselves, more than they would be to others. They often feel hopeless. These are hopeless thoughts. If I hadn't experienced this, I would have... There may be thoughts like, 'What would time be like?'" (P10)

The Psychological Symptoms theme comprises four categories: Panic Attack, Depression, Somatization, and Anxiety. The following texts include some of the views of training counselors regarding the symptoms of psychological trauma.

"The person feels the most guilt about what they couldn't do. I wish I had been there for them. A person experiencing loss says, 'I wish I had spent more time with them, I wish I had helped them with something.' In other words, the person grieves for the moments that weren't experienced when there were still things they could have experienced with that person. I often see people blaming themselves for this aspect." (P2)

"Because of the loss of trust, they can feel inadequate and helpless. This loss of trust can lead to depression. It can turn into social phobia. Generalized anxiety disorder can develop." (P5)

"The most common symptom I encounter in individuals who have experienced trauma is somatization. These people invariably experience feelings somewhere in their bodies. I give examples: very intense back pain, allergies, physical somatic symptoms, like a constant lump in their abdomen, a heaviness in their chest, a feeling of not being able to swallow in their throat... I also observe teeth clenching and very intense panic attacks. They are very intense physical symptoms. They affect the body. Because when repressed, intense emotions are not worked through and regulated, many things actually manifest as physical illnesses. We could even add cancer to this. This situation can even lead to cancer." (P6)

"They experience panic attacks, allergies, body aches, restless leg syndrome. Or they experience depression." (P8)

"Sometimes depression and panic attacks can accompany them. The need for acceptance and approval is often "These problems remain unresolved. Therefore, they need an external factor to accept and validate themselves. Their self-esteem is low. As a result, they experience self-confidence problems, which can be accompanied by depression and eating disorders for a long time. Anxiety disorders can also accompany despair." (P10)

The Course of Symptoms theme comprises four categories: Sudden Onset, Increase Over Time, Type of Trauma, and Personal Characteristics. In this context, some of the views of training counselors working with trauma on trauma symptoms are presented below. The following texts provide some of the views of training counselors regarding the progression of trauma symptoms.

"It can vary depending on when the traumatic event occurred. For example, if someone has recently experienced a traumatic event, they may be very tearful and rebellious. After some time has passed, they may become a little quieter." (P1)

"Their ability to disclose can vary depending on the traumatic event. For example, in cases of sexual abuse, a person tends to be more withdrawn and less open-minded. But I can communicate more easily with someone who has lost a parent, even in the early stages." (P2)

"The intensity of trauma is actually specific to the individual and the traumatic event. Sometimes the loss of a cat or bird can be very challenging for someone, while for another, it may not be so challenging. Or someone else gets into a serious car accident and escapes unharmed. This isn't a traumatic experience for them, but for someone else, it might prevent them from driving for years. Similarly, we can't expect someone bullied at school to have the same emotional intensity of traumatic reactions as someone who experienced sexual abuse. These depend on both the individual and the traumatic event." (P7)

"Crying fits can be seen as a reaction to trauma, especially in women. Men tend to experience crying fits more discreetly." (P8)

"There are natural and human-induced traumas. Human-induced traumas affect people more because they have difficulty making sense of them. For example, if an individual has a belief system, they might interpret a natural trauma as coming from God. By reconciling it in their minds in this way, they can feel more at ease. However, with human-induced trauma, a person can't find the cause. This way, they can't make sense of this trauma and can't place it in their mind. This affects the symptoms and severity of the trauma they experience." (P10)

The findings regarding the responses of the training psychological counselors to the questions about the post-traumatic recovery process are given in Table 3.

Table 3

Findings regarding responses of trauma-working psychological counselors to questions about the post-traumatic recovery process

Themes	Categories	Frequency	Percentage (%)
Stages of Recovery	Shock	6	%60
	Denial	4	%40
	Anger	8	%80
	Confrontation/Awareness	6	%60
	Acceptance	10	%100
Positive Influencing Factors	Communication Skills	6	%60
	Social Support	7	%70
	Emotional Regulation Skills	4	%40
Negatively Influencing Factors	Avoidance	7	%70
	Anxiety	4	%40
	Loneliness	5	%50
Social Support Systems	Positive Support Speeds Up the Process	8	%80
	Negative Support Slows Down the Process	5	%50
Individual 's Strengths	Intelligence Level	4	%40
	Social Support	6	%60
	Hobby/Sport	7	%70
	Psychological Resilience Level	4	%40
	Self-Compassion Level	5	%50
	Social Environment	3	%30
	Religion/Spirituality	5	%50
	Personality Traits	10	%100
Individual's Weaknesses	Parental Attitude	6	%60
	Personality Traits	7	%70
	Lack of Social Support	6	%60
	Loneliness	6	%60
	Negative Thinking	4	%40
	Ongoing Stress	4	%40

Table 3 presents the findings regarding the views of formator psychological counselors working with trauma, regarding the post-traumatic recovery process. Looking at Table 3, a total of 6 themes were identified: Recovery stages, Positive Influencing Factors, Negative Influencing Factors, Social Support Systems, Individual's Strengths, and Individual's Weaknesses.

In the Stages of Recovery theme, 5 categories were generated: Shock, Denial, Anger, Confrontation/Awareness, and Acceptance. The following texts include some of the views of training psychological counselors regarding the post-traumatic recovery process.

"I can generally say that because traumatized people shut themselves off from the event, the process feels unreal or like a dream, and their connection to reality is disconnected. They are still in the process of making sense of the events, not accepting them, and wondering whether they have a connection to reality and whether it was a dream. Because they don't accept this, they can't make sense of it. They are essentially in a state of shock." (P1)

"There are natural reactions that should occur in the natural selection of trauma. These reactions can vary depending on the individual's nature and perception. Because the first phase of trauma is generally denial,

anger outbursts can be seen more frequently." (P4)

"Therefore, I think people begin to accept themselves, then develop self-compassion, and then stabilize themselves to confront these traumas, and their ego strength increases. As ego strength increases, it becomes easier to look at the events there, or the events they experience become visible. Therefore, when a person looks at the present moment, they can see those feelings, emotions, and negative evaluations with a more realistic perspective, and their awareness of the event increases." (P6)

"The Process "First, uncovering anger—enabling the client to express their feelings without suppressing them—the traumatized person, then uncovering false belief patterns and thoughts, confronting them, and finally, allowing the person to accept the event from a different perspective—are the stages of the healing process." P(8)

"After they accept the process a little more, they begin to communicate more openly with the counselor. In other words, I can say they begin to establish a healthier relationship. Communication can be easier in later stages than it was in the early stages." (P9)

In the Positive Influencing Factors theme, 3 categories were generated: Communication Skills, Social Support, and Emotional Regulation Skills. The following texts include some of the views of training psychological counselors on the factors that positively affect the post-traumatic recovery process.

"Condolences, for example, are very important. They hold a very important place in our culture. When someone dies, everyone gathers at their home and supports the deceased's family. We are with you, you are not alone, we share your pain. In fact, this is the general purpose of condolence houses. We say, 'We are with you,' at least until the shock period or the acceptance period. Buying bread or bringing food, for example, is a demonstration of support. You can't deal with these things right now; these aren't your first needs, but we are supporting you. This is how we send them the message." (P6)

"The abundance and awareness of your inner resources are important. How do I define myself? For example, am I empathic? Am I stable? Am I strong? The more aware I am of these things, the higher my ego strength. The more I can regulate my emotions. If my emotional regulation capacity is at a good level, my healing process progresses more smoothly. If I can stay in the moment, I move forward comfortably. If I can't stay in the moment, the process progresses more slowly." (P7)

In the Negative Influencing Factors theme, 3 categories were generated: Avoidance, Anxiety, and Loneliness. The following texts present some of the views of training counselors regarding the factors that negatively impact the post-traumatic recovery process.

"Emotional symptoms can vary depending on the individual. For example, if they are more childish, communication is a little more difficult. These individuals can be more secretive. They become more anxious. I can say this especially for adolescents." (P2)

"If they are ostracized by society... In other words, if they are deprived of social support resources, if they have difficulty explaining a traumatic experience to society, the process is negatively affected by these factors." (P5)

In the Social Support Systems theme, 2 categories were generated: Positive Support Accelerating the Process and Negative Support Slowing Down the Process. The following texts present some of the views of training counselors regarding the impact of social support systems on individuals experiencing trauma.

"I think support is very important in trauma. Whether a child, adolescent, or adult, the better support they receive after trauma, the less severe the effects of trauma. The less it spreads. However, if there is no support and those around them ignore the trauma, the more severe the effects of trauma appear later, and the longer the impact lasts." (P1)

"I underlined a sentence in Şule Öncü's book, We Are All Narcissists. It says that people are the insurance of others. Indeed, it is. People need the presence of another person in good times and bad. In this sense, the stronger and more solid the social support sources that nourish a person externally, the easier it is for them to return to life." (P2)

"Negative social support systems affect traumatized individuals even more negatively. Positive social support systems also lead to a more positive outlook. They lead to a more manageable state." (P5)

"Here, social support needs to be positive. Otherwise, there is family, but it doesn't function properly; there are friends, but friends don't function properly. There needs to be a bond that functions properly. My family is behind me, they're there for me, but if it doesn't, if the family is inside me, it doesn't do much good. It does harm. Family isn't always good." (P9)

In the Individual's Strengths theme, 8 categories were generated: Intelligence Level, Social Support, Hobbies/Sports, Psychological Resilience Level, Self-Compassion Level, Social Environment, Religion/Spirituality, and Personality Traits. The following texts present some of the views of training counselors regarding the strengths of individuals experiencing trauma during the recovery process.

"If a child has a social circle, I would say it's actually one of their strengths. If they have friends, it could also be their social circle, or their family relationships. For example, if a child has a hobby, their hobby can make the process go more smoothly during the trauma process. In other words, a hobby helps the process. Painting, music... Maybe they don't continue a hobby during a trauma. We can't force this during the process. But if they have an active hobby, ensuring it's not interrupted and that they continue is one of their strengths." (P1)

"I read a book by Irvin Yalom. As far as we know, he doesn't have a religious belief or a belief in God. Yalom and his other colleagues, as an assistant, work in a hospital counseling people with cancer who are in the final stages, meaning they are hopeless. He often observes this: He says that people with a belief in God are more accepting of their situation and more prepared to face death; those who don't have this belief experience anxiety about it. He says this helps people find meaning in life, in the suffering they experience, and in the pain they experience." (P2)

"Being extroverted, having a strong ability to express oneself, having a high intellectual capacity, good psychological resilience, intelligence... I think it's very important to have studied at a good school or have a good education level, and sports. If someone participates in sports, they're stronger. Frankly, especially group sports." (P3)

"Religious belief or belief in God can be very effective in coping with trauma." (P6)

"If a person has good social support, it's easier to get through this process. Having a good, supportive partner, a circle of friends who stand by them, and people who understand them. Feeling the presence of others around them makes a person feel safe. The reason traumatic situations occur is because a person doesn't feel safe. This is where the environment comes into play. You're safe, we're with you." (P7)

"The level of self-compassion, the person's self-satisfaction, their capacity to interpret the world—in other words, their belief that they are good and the world is a just place. Furthermore, the individual's courage. Courage, I definitely think, is very valuable. Consistency, the continuity of an action, because therapy requires continuity. And patience. I would say the individual's patience." (P9)

"Everyone experiences a bad experience in their lives. Why does it cause trauma in some people and not others? Because there are no social support mechanisms. This is the real issue with trauma. Individuals shouldn't be left alone in the face of a bad experience, violence, abuse, or similar events. It's important to be able to talk to someone and have them help them. It's very important for an individual to say, 'I went through something like this, but someone supported me and believed in me.'" (P10)

In the Individual's Weaknesses theme, 6 categories were generated: Parental Attitude, Personality Traits, Lack of Social Support, Loneliness, Negative Thinking, and Ongoing Stress. The following texts present some of the views of training counselors regarding the weaknesses that hinder/obstruct the healing process of individuals experiencing trauma.

"A person has too many triggers, is always in an active environment. For example, living with someone who has a lot of family trauma. And those cycles repeat constantly." (P1)

"For example, a person is subjected to violence. They tell their mother, father, or the police. Their mother or the people they told didn't care about this individual. Let's say someone who was abused by their husband was left alone in this sense. They weren't cared about, their stories weren't taken seriously. In fact, leaving a person alone in these difficult experiences causes trauma." (P2)

"I can list some of these factors as being introverted, having a low IQ, not leaving the house, having little interaction with others outside of that, and having a low level of education, and poor self-care." (P3)

"If a person grew up in a neglectful family, meaning one where they were neglected, I can say that they have a harder time overcoming traumatic events." (P5)

"Unfortunately, neglected children have a harder time dealing with future traumatic events. This is a weakness for them. They start life 1-0 down." (P6)

Discussion, Conclusion & Suggestions

The purpose of this study is to examine in detail the views of training counselors working with trauma about trauma. The findings are presented in this section of the study. The results are discussed in light of the relevant literature.

When examining the views of trauma-related psychological counselors working with trauma, a total of 5 themes were identified: physical and behavioral symptoms, emotional symptoms, cognitive symptoms,

psychological symptoms, and the course of symptoms. Accordingly, when looking at the theme of physical and behavioral symptoms, 13 categories were generated: headache, stomach pain/nausea, avoidance, sleep disorder, nail biting, incontinence, tics, hyperactivity, attention deficit, unwillingness, dullness, anorexia, and aggression. These symptoms overlap with the diagnostic criteria for post-traumatic stress disorder, which include marked physiological responses to stimuli that remind one of the traumatic event, aggression, sleep disturbance, constant vigilance, a significant decrease in interest in participating in important activities, and persistent avoidance of stimuli related to the traumatic event (APA, 2014). The literature also supports this information. For example, in a qualitative study conducted with Syrians under temporary protection status, it was found that trauma caused psychosomatic symptoms such as bodily pain and complaints in the vast majority of participants. Furthermore, it was observed that participants appeared isolated from the social structure and felt excluded (Karacoşkun & Kaya, 2025). Similarly, in a qualitative study conducted by Gören (2023) with individuals who followed the February 6, 2023 Kahramanmaraş earthquake through the media, sleep disturbance, hyperarousal, and symptoms of meaninglessness and detachment from routine were revealed among secondary traumatic stress responses. In this respect, the views of trainer psychological counselors regarding physical and behavioral trauma symptoms are consistent with the relevant literature.

When looking at the theme of emotional symptoms; Eight categories were generated: anger, sadness, guilt, inadequacy, helplessness, worthlessness, hopelessness, and regret. Likewise, the diagnostic criteria for post-traumatic stress disorder symptoms in DSM-V include persistently experiencing negative emotional states such as fear, horror, guilt, rage, or shame (APA, 2014). Because traumatic experiences usually occur unexpectedly, individuals may experience feelings such as anxiety, fear, helplessness, and vulnerability (Papanikolaou et al., 2011). In the psychoeducation program on challenging life events prepared for teachers by the Ministry of National Education (2024), feelings of fear, anxiety, anger, helplessness, hopelessness, regret, worthlessness, guilt, and numbness are included among post-traumatic emotional responses. In this context, in a study where the participants were teachers, a sub-dimension of emotional symptoms including elements such as isolation and loss of social support resources was identified in the measurement tool developed to evaluate the post-traumatic stress responses of preschool students after the earthquake (Akyüz Uçar & Çetin, 2024). It is seen that the teacher data obtained are consistent with the observations of the trainer psychological counselors. Accordingly, it can be said that the category of emotional symptoms is consistent with the relevant literature.

When looking at the theme of cognitive symptoms, 6 categories were identified: difficulty in decision-making, forgetfulness, inability to focus, rumination, self-blame, and confusion. In this direction, the diagnostic criteria for post-traumatic stress disorder symptoms in DSM-V include negative changes in cognitions related to the traumatic events. For example; These include symptoms such as inability to remember a significant aspect of the traumatic event, negative beliefs such as “Nobody can be trusted,” distorted cognitions that cause the individual to blame themselves or others, difficulty concentrating, involuntary distressing memories of the traumatic event, recurrent dreams, a feeling as if the events are recurring, or dissociative reactions (APA, 2014). The Ministry of National Education (2024) psychoeducation program also includes post-traumatic cognitive responses such as forgetfulness, memory problems, loss of self-confidence, and difficulty in decision-making. Therefore, it can be said that the observations of the trainer psychological counselors are consistent with the relevant literature.

Regarding the psychological symptoms theme, four categories were emerged: panic attack, depression, somatization, and anxiety. In a study conducted by Ayas (2005) after the August 17, 1999 Marmara earthquake, psychological symptoms such as depression, anxiety, psychoticism, phobic anxiety, eating and sleep disorders, somatic and obsessive-compulsive disorder were observed and found to differ according to certain variables. Similarly, a study conducted with cancer patients revealed that individuals with symptoms of post-traumatic stress disorder had significantly higher stress, anxiety, and depression scores (Erdoğan, 2015). Gluckman (2012) states that examples of psychological reactions observed after an earthquake include confusion, replaying the earthquake moment in one's mind, difficulty falling asleep, guilt, outbursts of anger, fear, emotional depression, and strong emotions such as anxiety. Similarly, Bekaroğlu and Yılmaz (2020) stated that the Covid-19 pandemic led to psychological symptoms such as anxiety, fear, anger, depression, insomnia, and post-traumatic stress disorder. In a study conducted in China by Wang et al. (2020), 53.8% of participants stated that the pandemic negatively affected their psychology. In addition, 16.5% of participants reported experiencing moderate-to-severe depressive symptoms, 28.8% experienced moderate-to-severe anxiety, and 8.1% experienced moderate-to-severe stress. In this respect, the opinions of trainer psychological counselors regarding psychological trauma symptoms are consistent with the relevant literature.

When examining the course of trauma symptoms, four categories were identified: sudden onset, increase over time, type of trauma, and personal characteristics. According to the opinions of training psychological counselors who work with trauma, the course of trauma symptoms can manifest as sudden onset and increase over time. Furthermore, the course of trauma symptoms can vary depending on the type of trauma and the individual's personality traits. There are two distinct periods following traumatic events: the acute period and the post-traumatic period (Tanhan & Kayri, 2013). The acute period encompasses the first thirty days after an individual experiences a traumatic event (Bayraktar, 2012). The post-traumatic period encompasses a period longer than a month after the traumatic event (APA, 2014). In most individuals, symptoms of post-traumatic stress disorder begin within hours or days of confronting the trauma. However, in some cases, symptoms can emerge years later. For example, individuals who survived a concentration camp repressed all trauma-related memories immediately after the war and entered an intensive work schedule. It has been observed that when workload decreases or ends, post-traumatic stress symptoms persist and they experience numerous difficulties adapting to social life (Marmar et al., 1996). Trauma symptoms have been reported to develop differently in different individuals depending on the type of traumatic event experienced, individual differences, past life trauma history, temporal distance to the traumatic event, and certain variables that develop before, during, or after the trauma (Maes et al., 1998). For example, it has been stated that human-induced and intentional traumatic experiences have a higher risk of causing post-traumatic stress disorder (Karancı et al., 2009). Similarly, according to a meta-analysis examining traumatic events under two headings: human-induced and nature-induced trauma, the prevalence of post-traumatic stress disorder within the first year of life for individuals experiencing human-induced trauma is found to be between 25-75%, while this rate decreases to 5-60% for nature-induced traumatic events (Galea et al., 2005). Therefore, this finding is parallel to the results obtained in our study. In conclusion, the views of trainer psychological counselors regarding the course of trauma symptoms are consistent with the literature. It can be said that in the evaluation of trauma symptoms, variables such as personal characteristics and type of trauma should be considered together with temporal factors.

When the views of the training psychological counselors working with trauma about the post-traumatic recovery processes were examined, a total of 6 themes were identified: recovery stages, positive influencing factors, negative influencing factors, social support systems, individual strengths and individual weaknesses. The recovery stage theme revealed five categories: shock, denial, anger, confrontation/awareness, and acceptance. While the post-traumatic recovery process is explained with different models in the relevant literature, it fundamentally involves the process of making sense of the traumatic experience, reestablishing emotional balance, and reconnecting with life. According to [Herman \(2017\)](#), this process consists of three stages. The first stage is the safety stage. In this stage, the priority is to ensure the physical and psychological safety of the traumatic experience and to help the person regain lost control. The second stage is the remembering and grieving stage. This stage involves the individual making sense of the trauma by narrating the traumatic event and sharing their feelings. The final stage is the reconnecting stage. In this stage, the individual regains lost control, improves self-esteem by recognizing their strengths, develops an optimistic outlook, increases hope, and gains an understanding of how to use new coping skills for potentially challenging experiences. Similarly, [Kübler-Ross's \(1969\)](#) stages of loss and grief are comprised of five stages: denial, anger, bargaining, depression, and acceptance. In the denial stage, the individual questions whether the loss is real. In the anger stage, the individual experiences intense anger. In the bargaining stage, they consider what they could have done differently, along with feelings of guilt and regret. The depression stage is the stage in which the individual accepts the death. The acceptance stage is the stage in which the individual can now see the truth and reorganize themselves accordingly. Consequently, both models in the literature highlight the process of making sense of the traumatic experience, achieving emotional balance, and reconnecting with life. Accordingly, in this study, the post-traumatic recovery process consists of five stages: shock, denial, anger, confrontation/awareness, and acceptance. The stages of shock and denial explain the initial impact of the traumatic experience and the individual's difficulty accepting reality. The stage of anger describes the period of intense emotional release. The stage of confrontation/awareness represents the stage in which the individual confronts the traumatic experience and begins to make sense of it. The final stage is acceptance. This stage represents the process in which the individual internalizes the traumatic experience, reorients their life, and regains emotional balance. Therefore, the categories established for the stages of post-traumatic recovery in this study can be explained consistently with models in the literature. Accordingly, the views of trainer psychological counselors regarding the stages of recovery are consistent with the relevant literature.

The findings of the study indicate that social support plays a dual role in post-traumatic recovery. Social support, on the one hand, is a factor that positively impacts the recovery process and is among the individual's strengths; on the other hand, lack of support, negative support, and loneliness are among the themes that negatively impact recovery and are among the individual's weaknesses. This suggests that social support is not only a protective factor but also a risk factor when it is dysfunctional or absent. In this respect, the findings of the study indicate that the quality of social support has a significant impact on the post-traumatic recovery process. The relevant literature is consistent with these findings. Social support is a multidimensional construct that provides individuals with material and social support from their environment. It is also a factor that helps balance feelings of loneliness and helplessness caused by the loss of control that occurs in the individual after experiencing trauma ([Rodriguez & Cohen, 1998](#)). Furthermore, sharing experiences with support groups can help individuals exposed to a traumatic event make sense of the trauma by making them feel they are not alone

(Joseph & Linley, 2006). The quality of social support becomes crucial in this regard. The relevant literature indicates that victims with more supportive relationships recover faster and exhibit fewer post-traumatic symptoms than those without supportive relationships (Brewin et al., 2000). Similarly, a study conducted by Shang et al. (2022) following the Lushan earthquake in China revealed that individuals who received social support and perceived this support as high quality showed more signs of post-traumatic growth, while those who described the quality of their perceived social support as low showed less signs of post-traumatic growth. Accordingly, the views of trainer psychological counselors regarding the effect of social support on the post-traumatic recovery process are consistent with the relevant literature.

The findings of the study indicate that communication skills and emotion regulation are among the factors that positively impact the post-traumatic recovery process. Cognitive processes such as self-expression and emotion-regulating activities are important predictors of post-traumatic growth (Forgeard, 2013). It has been stated that people who can communicate effectively can solve problems more easily (Işık, 2018), develop satisfying relationships, and be more successful in their social lives (Cüceloğlu, 2003). Furthermore, it has been stated that regulating emotions helps individuals develop effective coping strategies by maintaining psychological balance in the face of stressful events (Gross, 1998). Furthermore, research has revealed that people who use effective emotion regulation skills have lower anxiety levels, fewer depressive symptoms, and higher levels of satisfaction in their social relationships (Aldao et al., 2010). In conclusion, it can be said that communication skills and emotion regulation skills are among the protective factors that positively affect the post-traumatic recovery process. This finding is parallel to the results in the relevant literature. Furthermore, another finding of the study, regarding the factors that negatively affect the recovery process, reveals three categories: avoidance, anxiety, and loneliness. Individuals exposed to traumatic experiences often exhibit avoidance (Aker, 2012; APA, 2014) and experience feelings of anxiety (Ferguson et al., 2008). Furthermore, when faced with negative events in their daily lives, people can react with social isolation, self-criticism, and self-centeredness (Germer, 2009). Research shows that avoidance (Schuettler & Boals, 2011), anxiety (Sarman & Tuncay, 2024), and loneliness (Palgi et al., 2021) are significantly associated with post-traumatic stress disorder. Therefore, avoidance, anxiety, and loneliness can be considered risk factors that negatively impact the post-traumatic recovery process. Thus, this finding of the study is consistent with the relevant literature. Accordingly, the views of trainer psychological counselors regarding factors that positively and negatively affect the post-traumatic recovery process are consistent with the relevant literature.

In the post-traumatic recovery process, an individual has both strengths and weaknesses that can negatively affect the process. When the personality trait factor, which is one of the factors affecting the post-traumatic recovery process, is examined, it is seen that personality traits are included in the theme of both an individual's strengths and weaknesses. This shows that personality traits have a protective role in the post-traumatic recovery process, while at the same time it shows that they have an effect that complicates the recovery process. For example, a positive personality trait such as extraversion plays a protective factor for post-traumatic stress disorder (Madamet et al., 2018), while introversion and high neuroticism pose a risk for post-traumatic stress disorder (Resnick et al., 1989). According to the findings of a recent meta-analysis that included forty studies, it was determined that the traits of mobility, sensory sensitivity, resilience, and activity were associated with a reduction in the symptoms of post-traumatic stress disorder; while emotional reactivity and perseveration were associated with an increase in the symptoms of post-traumatic stress disorder (Cyniak-Cieciura & Zawadzki,

2021). The studies; Studies have shown that personality traits such as neuroticism (Dai et al., 2017; Mennicken et al., 2022; Ogle et al., 2017), perfectionism (Ma et al., 2020; Price et al., 2020), and impulsivity (Morris et al., 2020; Netto et al., 2016) pose a risk for post-traumatic stress disorder, while personality traits such as openness to experience, agreeableness, and conscientiousness are positively correlated with post-traumatic growth (Tedeschi & Calhoun, 2004). Accordingly, the views of trainer psychological counselors regarding the impact of an individual's personality traits on the post-traumatic recovery process are consistent with the relevant literature.

When examining the factors influencing the recovery process after trauma, the findings of the study indicate that intelligence level, social support, having hobbies/sports, psychological resilience, self-compassion, religion-spirituality, and personality traits are among the individual's strengths that positively affect the post-traumatic recovery process. The studies conducted; Studies show that the level of psychological resilience (Connor et al., 2003; Weiss, 2002), self-compassion (Barlow et al., 2017; Yelpeze, 2020), social support (Joseph & Linley, 2006), intelligence level (Önder & Gülay, 2008), social environment (Cryder et al., 2006; Ergün Başak, 2012), sports (Alpullu & Yılgin, 2024), having a hobby (Aker, 2012; Yıldırım & Dilekmen, 2025) and religion/spirituality (Elçi, 2004; Önen & Kızılgeçit, 2024; Seo & Lee, 2020; Tedeschi & Calhoun, 2004) have a positive effect on coping with stressful and traumatic life events. On the other hand, according to other findings of the study, parental attitudes, personality traits, lack of social support, loneliness, negative thinking, and ongoing stress are among the individual's weaknesses that slow down or complicate the post-traumatic recovery process. Research has shown that negative parental attitudes (Beşer et al., 2019; Haapasalo & Pokela, 1999), loneliness (Palgi et al., 2021), lack of social support (Öztürk & Derin, 2020; Pedersen, 2015), negative thinking (Çakır et al., 2014; Ehlers & Clark, 2000), and ongoing stress (Özgen & Aydın, 1999) have a negative impact on coping with stressful and traumatic life events. In conclusion, it can be said that an individual's strengths support the recovery process, while their weaknesses slow it down. The key here is for an individual to recognize and actively utilize their strengths, while also recognizing and working to transform their weaknesses. This allows for a healthier post-traumatic recovery process. Accordingly, the views of trainer psychological counselors regarding the strengths and weaknesses of individuals that affect the post-traumatic recovery process are consistent with the relevant literature.

Limitations

This study was conducted with 10 psychological counselors who received trauma training and work in public institutions. Because the study was limited to a specific sample and subject area, the generalizability of the findings is limited. Furthermore, because the data were based on the participants' subjective experiences, recall bias or personal interpretations may have influenced the interview content. Therefore, these limitations should be taken into consideration when interpreting the study's findings. Based on the findings of the study, training programs can be developed to improve the skills of trainer psychological counselors in recognizing trauma symptoms and post-traumatic recovery processes. Furthermore, future qualitative research with more participants and in different contexts could increase the generalizability of these findings.

Ethic

This study was reviewed by the Necmettin Erbakan University Social and Human Ethics Committee. It was approved by decision number 2025/986 dated November 14, 2025.

Author Contributions

This study was prepared with the joint contributions of both authors.

Conflict of Interest

The authors declare that they have no conflict of interest regarding this study.

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